

Department of Health and Human Services
Assistant Secretary for Technology Policy and the Office of the National Coordinator for Health Information Technology
Attention: HHS Health Sector AI RFI
7033A, 330 C Street SW, Washington, DC 20201

February 23, 2026

RE: *Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care*

Dear Dr. Keane,

On behalf of IMPaCT Care (IMPaCT), we thank you for the opportunity to offer comments in response to the Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care.

We commend HHS' efforts to accelerate adoption of AI within care and advance federal incentives to enable AI to boost productivity, lower costs, and improve health outcomes. We believe there are significant ways that AI can support Community Health Workers in extending the reach of care quickly and at scale.

[CHWs](#) are trustworthy people who come from within the communities they serve. They get to know their patients as people and provide [holistic, tailored support](#) based on individual needs and preferences. Evidence-based CHW programs [improve outcomes, reduce costs,](#) and address the twin crises in American health care today: the growing prevalence of chronic illness in the context of clinical workforce shortages.

[IMPaCT](#) is the leading evidence-based Community Health Worker intervention in the U.S., designed to address the root causes of poor health. It is used by over 70 organizations across 20 states and has been found in multiple randomized controlled trials to improve [chronic disease control](#), [mental health](#), and [quality of care](#) while [reducing total hospital days](#) and [returning \\$2.47 for every dollar invested](#). In this letter, we offer comments in response to specific questions included in the Request for Information including how equipping CHWs with AI can expand access to person-centered and cost-effective care. We also recommend CHW technology tools that can increase access and improve quality and outcomes in the face of clinical workforce shortages.

Comments on Specific Questions included in the Request for Information

1. What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

The biggest barrier to AI adoption isn't an implementation or regulatory barrier; it's a failure of imagination. Most AI tools in health care today are aimed at medical providers (e.g., improving clinical decision support, reducing documentation burden). Building tools for health care providers can make them more efficient and reduce medical errors, but it is unlikely to fundamentally transform health outcomes for two reasons.

- First, there are simply not enough providers. The [average wait time for a physician appointment in large metro areas is 31 days](#), with even longer wait times for certain specialties (e.g., 42 days for an obstetrician-gynecologist). Behavioral health professionals are even more scarce - [fewer than half of people with mental illness get timely access to care](#), and even fewer people with substance use disorder are able to access the care they need. Challenges accessing care are even greater in rural areas and for individuals with Medicaid coverage.
- Second, health care providers are ill-equipped to address the root causes of poor health. Most of the American workforce are clinicians e.g., (physicians, nurses, medical assistants) whose main lever is medical care. Yet, medical care –the office visit, the hospital stay– only [accounts for 15% of what drives health outcomes](#). Eighty-five percent of health is shaped by behavioral, social, and economic factors.

AI can do more than buttress a broken system. It can unlock human connection and enable the health care workforce of the future. Community health workers (CHWs) are trustworthy individuals who come from within the communities they serve. They provide social support, care coordination, navigation and advocacy. Across four randomized controlled trials and a large Patient-Centered Outcomes Research Institute (PCORI) multi-site study, the IMPaCT model has been shown to save Medicaid payers [\\$2.47 per dollar invested](#), [improve mental health](#), [strengthen primary care engagement](#), [reduce 30-day readmissions](#) and [emergency department visits](#), and [cut hospital days by 34%](#) versus matched controls.

Importantly, CHWs achieve these outcomes at a fraction of clinical labor costs. CHWs represent an untapped source of human capital in health care, because they can be rapidly deployed in any community.

2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.

A primary challenge for AI-powered Community Health Worker models is that the majority of health care payment requires a clinical encounter with a health care provider. Several Center for Medicare and Medicaid Innovation models, including the Long-term Enhanced ACO Design (LEAD) model and the Geo track within the Achieving Healthcare Efficiency

through Accountable Design (AHEAD) model, create promising mechanisms for expanding community-based models of care through flexible payment and multi-payer alignment.

Nowhere is innovation more necessary than in the safety net. We encourage the Health Resources and Services Administration (HRSA) and the Centers for Medicare and Medicaid Services (CMS) to partner to facilitate inclusion of Federally Qualified Health Centers (FQHCs) in these models, including HRSA alignment to help FQHCs embrace AI.

6. Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?

Equipped with the right technology, Community Health Workers (CHWs) can be a new “front door” to health care that is more person-centered, less expensive, and more effective. In the IMPaCT model:

- CHWs use conversation prompts to solicit client goals. An AI scribe helps CHWs focus on the person in front of them, rather than on capturing notes.
- CHWs then leverage decision supports and workflow tools within the IMPaCT application that help them support goal attainment. The most common areas that clients need support with are psychosocial (37%), health behavior change (36%), and resources for daily living (e.g., housing, food; 15%). AI can help CHWs suggest resources, fill out benefits applications, and manage follow-ups over an average of 12 encounters with a client.
- When clients do need medical care and health system navigation, CHWs can use technology to rapidly connect to care teams to address common requests like prescription refills without a new visit to a provider.

Using these technology tools, CHWs are able to help clients achieve more than 70% of their goals, whether those goals are clinical (e.g., get my diabetes under control, find a behavioral health provider) or social (e.g., get the home modifications I need to live independently, apply for Social Security Income). AI acts as a force multiplier, allowing CHWs to more rapidly connect with clients and help them achieve their goals, all while managing larger panel sizes due to AI-driven efficiencies. By addressing a client’s greatest needs, CHWs generate durable changes in health, increase access, and improve quality while better using scarce clinician resources.

9. What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

More than half of American adults have multiple chronic conditions and are not well-served by our fragmented delivery system. The most [effective interventions are person-centered](#)

[care plans](#) that help individuals achieve their own goals. NCQA has developed [person-centered goal attainment quality measures](#), designed to measure what matters most to each individual. Use of these measures could realign care, but to date, they have been administratively burdensome to collect. AI tools can help efficiently capture person-centered goals, allowing the health care system to align around what matters most to patients.

Conclusion

Thank you again for the opportunity to provide comments. We look forward to continuing to collaborate with HHS on these critical issues. For any questions or to request more information, please contact Dawn Alley at dalley@impactcarehq.com.

Sincerely,

Shreya Kangovi, MD
CEO, IMPaCT Care